BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00464 v3: current regimen source · HPRA Dacarbazine 200 mg product information, March 2024

THE WHOLE TREATMENT JOURNEY

Dacarbazine — metastatic melanoma: treatment and checks

Advanced melanoma

Dacarbazine by IV infusion on day 1, repeated every 21 days for up to six cycles, or stopped earlier for progression or unacceptable toxicity.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00464	Full blood count, kidney and liver function, medicines and prior treatment history. Discuss pregnancy prevention and fertility before the first dose. Record symptoms and the disease sites being followed on scans.	Full blood count, kidney and liver function before every cycle. Blood-count suppression can be delayed: the product information describes the lowest counts often three to four weeks after a dose, so a 21-day appointment does not guarantee recovery. Extra blood tests are arranged when clinically needed.	Follow your unit's fever threshold and urgent-contact instructions; call immediately if you develop fever, shaking chills or feel suddenly unwell, even without a high temperature. Also report unusual bleeding, persistent vomiting, inability to drink, reduced urine, jaundice or severe abdominal pain. Fever with abdominal pain and jaundice needs urgent assessment for rare severe liver injury. Tell the nurse immediately about infusion-site burning or swelling.	Keep reporting fever, bleeding, worsening fatigue or other symptoms after the last infusion while counts and organs recover. Response scans, later blood tests and the next treatment decision are individual; the NCCP source gives no universal imaging or tumour-marker schedule.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
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What each check is for

1 Treatment visits

21-day reference cycle

To control metastatic melanoma with chemotherapy, with benefit, blood counts and tolerability reviewed before each treatment.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00464

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42